

Twinkle Children's Hospital

User guide

For the front desk, the doctor, and the pharmacy counter.

Every screenshot is taken from the running application. To refresh them all after a change, run the one test that produces them:

```
dotnet test tests/Pharma.UiTests --filter ScreenshotCapture
```

Contents

Getting started

1. The window
2. Settings — set this up first

The OPD desk 3. The OPD screen 4. Booking a visit 5. Taking the consultation fee 6. The consultation

The pharmacy 7. Medicines — the catalogue 8. Inventory — what is on the shelf 9. Importing a supplier bill 10. The pharmacy counter

Everything else 11. Patients 12. Reports 13. Printing 14. Worked examples 15. What the system will and will not do 16. When something goes wrong

Common tasks, step by step — the short version of everything above. Start here if you just want to get through a day.

Common tasks, step by step

Each task below is complete on its own. Screen-by-screen detail follows in sections 1 to 16.

A. Set the clinic up — once, before anything else

1. Click **Settings**.
2. Fill in **clinic name**, **address**, **phone**.
3. Tick **Registered for GST** only if the clinic actually is. Then enter the **GSTIN**. Leave it unticked and bills print as a plain invoice.
4. Enter the **drug licence number** and the **pharmacist's name**.

5. Click **Save Clinic / Pharmacy details**.
6. On the right, enter the first **doctor** — name, speciality, registration number, usual fee — and click **Save doctor**.

You cannot book a visit until at least one doctor exists.

B. Book a walk-in patient

1. Click **OPD**, then **+ New visit**. The booking form opens over the screen.
2. Type the patient's **name or phone number** and press Enter.
3. If they are listed, **click the right person**. If a whole family shares the phone, everyone on it appears — pick the child who is actually here.
4. If nobody matches, the new-patient form opens. Enter **name, phone, age, sex**.
5. Choose the **doctor**, adjust **time** and **fee** if needed.
6. Type the **complaint** if you want it on the tile and the prescription.
7. Click **Book visit**. A token number is allocated.

C. Take the consultation fee

1. On the **OPD** screen, find the patient's tile in **Waiting**.
2. Click **Fee** on the tile. A small form opens over the screen.
3. Check the **token, name, age and doctor** at the top — this is the moment to notice you have the wrong child.
4. The **fee** is already filled in from the booking. Change it if you are taking something else — a follow-up at half fee, a family concession.
5. Set **Paid by** — Cash, UPI or Card.
6. Leave **Print the receipt** ticked unless you do not want paper.
7. Click **Take fee**. It asks once more, naming the amount and the child. Answer **Yes**.

The badge changes to **Fee paid**. Clicking Fee twice does nothing.

Nothing is written until you answer Yes. Cancel at any point and no receipt number is used up. This matters because a receipt is numbered and dated the moment it is written — a fee taken wrongly has to be reversed on paper, so the software makes you look at it first.

D. See a patient and write a prescription

1. Click **Consult** on the patient's tile.
2. Fill in **vitals, complaint, diagnosis, advice** — all optional.
3. For each medicine:
 - Type two letters or more into **Medicine**.
 - **Click a match** to use one you stock, or keep typing for one you do not.
 - Enter the **dose**.

- Set **Morning · Afternoon · Night** from the three lists — **0** , **1/4** , **1/2** , **1** or **2** . One in the morning and one at night is **1** , **0** , **1** .
 - Enter **days**.
 - **Qty is filled in for you** in individual tablets — change it if you want.
 - Click **Add**.
4. Set a **review date** if there is one.
 5. Click **Save & complete**. The tile moves to **Completed**.

The consultation covers the whole window while it is open, and the rest of the app waits behind it. Nothing else can be started until you leave, so a consultation cannot be opened and then forgotten. **Close** or the **Esc** key leaves it; if anything has been typed and not saved you are asked first.

E. Add a new medicine and put stock on the shelf

This is two jobs on two screens. **Medicines** is where a medicine is described, once. **Inventory** is where stock arrives, every delivery.

First, describe the medicine — Medicines screen

1. Click **Medicines**, then + **New medicine**. The form opens over the screen. To change one already there, select the row and click **Edit** — or just double-click it.
2. Enter the **brand name** as printed on the pack — *Calpol*.
3. Enter the **drug / generic name** — *Paracetamol 250mg*. Staff search by either, so filling both is worth the few seconds.
4. Enter the **manufacturer** and **pack size** as printed.
5. Set **Sold as** — tablet, capsule, bottle, sachet.
6. Set **Units in one pack** — **15** for a strip of fifteen, **1** for a bottle.
7. Set **GST %**, **schedule**, **rack** and **reorder level**.
8. Click **Save medicine**. The form closes — that is normal. The message underneath the catalogue confirms what was saved.

Then, put stock on the shelf — Inventory screen

9. Click **Inventory**, type the name in the search box and click **Search**.
10. Click the medicine in the list. The heading names it and says how much is on hand.
11. Click **Receive stock**. A form opens over the screen, headed with the medicine's name and what is already on the shelf.
12. Enter **batch no**, **expiry**, **packs**, any **free** packs, the **rate** you paid and the **MRP** printed on the pack.
13. Check the line that appears — *"20 pack(s) × 15 = 300 tablets onto the shelf"*.
14. Click **Add stock**. The form closes and the screen clears, ready for the next line of the delivery note.

If you are entering several medicines off one delivery note, repeat steps 9–14 for each.

Everything clears between lines, the supplier and bill number included. That is deliberate. A supplier left in the box is a supplier silently attached to the next delivery, and a wrong supplier on a batch is worse than a blank one — it is wrong in the reconciliation report rather than merely missing from it.

F. Load a supplier's file instead of typing it

1. Click **Inventory**, then **Import supplier bill**.
2. Choose the **supplier profile** that matches their format.
3. Click **Browse...** and pick the file. It is read immediately.
4. Type the **supplier name** — the file does not contain it.
5. Check the grid, especially **PER PACK**. `30s` reads as 30; `60ML` stays 1. **Type the real number for any strip.**
6. Read the notices underneath — bill date, MRP changes, anything unclear.
7. Click **Import**.

Stock is **added** to what is already there. The same bill cannot be imported twice.

G. Sell to someone who walks in

1. Click **Pharmacy counter**.
2. Type part of the medicine name. It filters as you type, in any case.
3. **Click the medicine** in the results.
4. Enter **How many**, and check the unit beside it says what you mean — *tablets* or *strips of 10*.
5. Click **Add to bill**. The search box empties and the medicine is let go of — the line is on the bill, so start the next one by typing its name.
6. Repeat 2–5 for each medicine. The bill builds up; only the search clears.
7. Leave the customer as **Cash**, or type their name.
8. Choose **Payment**, then **Save & print bill**.

H. Sell part of a strip

Exactly as task G. Nine tablets out of a strip of ten is `9` with the unit on **tablets** — the bill line then reads `0 x 10 TAB + 9 tablets` so you can see what to hand over.

The price is proportional: a strip of ten at ₹120 makes each tablet ₹12, so nine is ₹108. **A full strip always costs exactly the MRP printed on it** — the arithmetic never drifts by a few paise.

Switch the unit to **strips of 10** and type `2` when someone wants whole strips; it becomes 20 tablets.

If the unit only offers "units"

That medicine has **Units in one pack** set to 1, so the software believes one strip is one tablet — and nine tablets will be charged as nine strips. **Settings** → **Check data health** finds every medicine in that state and fixes them together. See task N.

I. Dispense what a doctor prescribed

1. Click **Pharmacy counter**.
2. Under **Or pull today's OPD prescription**, choose the patient.
3. Click **Load prescription**. Everything prescribed **and in stock** is added with the right quantity.
4. Anything missing is named on screen — tell the parent to buy it outside.
5. Choose **Payment**, then **Save & print bill**.

J. Correct a stock count

1. Click **Inventory** and select the medicine.
2. Click **Correct count**. A form opens over the screen.
3. Choose the **batch**. If there is only one it is already chosen.
4. Enter the **true count** — what is actually on the shelf, in units. It starts at what the system currently believes, so you are changing a number rather than typing one from nothing.
5. Choose a **reason**: recount, breakage, expired, lost, entry error.
6. Add a **note** if it is unusual.
7. Click **Correct count**.

Every correction is recorded in **Recent corrections** at the foot of the Inventory screen, with was, now and why.

If the medicine has nothing on the shelf, it says so rather than opening an empty form — there is no count to put right.

The form closes afterwards and takes the batch with it. Correcting a second batch means opening it again — which is the point: a correction made against a batch you had forgotten was still selected writes off the wrong stock, and the trail then says you meant to.

K. Print something again, however long ago

1. Click **Patients** and search by name or phone.
2. Select the patient.
3. **Visits & prescriptions** tab → select the visit → **Print prescription** or **Print fee receipt**.
4. **Medicine bills** tab → select the bill → **Print bill**.

For a walk-in with no patient record: **Reports** → **Day book** → **Find any bill**, search the bill number or customer name across all dates, then **Reprint**.

L. Close the day

1. Click **Reports**.
2. Check the totals along the top — pharmacy sales, cash, UPI, fees collected.
3. **Day book** — every bill of the day.
4. **GST summary** — only if registered.

5. **Expiring soon** — return these to the distributor.
6. **Low stock** — what to order.
7. **Stock to reconcile** — anything added at the counter without a supplier bill. Match each to the bill when it arrives.
8. **Schedule H1 register** — keep for three years.

M. Sell something the system says you do not have

1. At the **Pharmacy counter**, find the medicine and click it.
2. Click **Stock came in — add it**.
3. Enter **packs on the shelf** and the **MRP**. Everything else is optional.
4. Click **Add to shelf** and carry on with the bill.

It appears under **Reports** → **Stock to reconcile** until you match it to the supplier bill.

N. Put the medicine records right

Do this **once**, before you rely on the counter. It is the difference between nine tablets costing ₹108 and costing ₹1,080.

1. Click **Settings**, then **Check data health**.
2. Read the list. Each row says what is wrong, what it will become, and what happens to the stock figure.
3. Leave the ticks as they are and click **Put the ticked ones right**.

What it finds	What it means
Pack size disagrees	The pack says 15 but the medicine says 1 per pack, so the counter sells whole strips to anyone asking for tablets. Fixing it re-counts 59 strips as 885 tablets — the same packs on the shelf
Old stock at a different pack size	Stock received before the medicine was corrected. Re-counted to match
Sold-as not set	Quantities read "units" instead of tablets. Taken from what is printed on the pack
Duplicate — fix by hand	The same medicine twice. Press Merge... on the row

Every re-count writes a line in **Inventory** → **Recent corrections**, so nothing changes quietly.

Before you tick a re-count, check one thing

"59 → 885" is right **only if the 59 means 59 strips**. If it means 59 loose tablets, untick that row — it would multiply your stock by fifteen.

O. Merge a medicine that appears twice

1. **Settings** → **Check data health**.
2. Find the row marked **Duplicate** and click **Merge...**
3. Confirm.

Its batches, purchases, sales and prescriptions all move to the record holding the most stock, and the empty one is retired. Nothing is deleted.

You will not be able to create a new duplicate — the same brand, maker and pack is refused, and you are offered the existing one instead.

1. The window

Seven screens down the left. The one you are on is highlighted.

Button	What it is for
OPD	The day's queue. Book visits, take fees, open consultations
Patients	Everyone ever registered, with their whole history
Pharmacy counter	Selling medicines
Medicines	The catalogue — what each medicine <i>is</i> . Set up once
Inventory	Stock — what is on the shelf. Receiving, batches, corrections
Reports	End of day, GST, expiry, low stock
Settings	Clinic details, doctors, screen layout

Medicines and Inventory are deliberately separate. Describing a medicine is a one-off job usually done by whoever sets the shop up; receiving stock happens every delivery and is done at the counter. Keeping them apart means neither screen is crowded with the other's fields.

The application opens **maximised**, filling the screen. The screens it runs on are small, and every pixel spent on desktop around the edge is a pixel not spent on the queue.

Forms open over the screen

Anything you fill in opens **over** the screen you were on, greyed out behind it, and the rest of the app waits until you are done:

- Booking a visit, and taking the fee
- Adding or editing a patient
- Adding or editing a medicine

- Receiving stock, and correcting a count
- The consultation, importing a supplier bill, a print preview

This is on purpose, and it is worth knowing why. These used to be columns down the right-hand side of each screen, permanently there and never empty. On a small screen that meant scrolling to reach the Save button — and a form you scroll to save is one people abandon half-filled. Worse, a column that is never empty always has *something* in it from the last time, which is how a patient gets saved on top of another one.

A form over the screen is opened for one job and taken away when that job is done. **Cancel**, or the **Esc** key, closes it and writes nothing.

That is also why the lists behind them now have the whole window: the catalogue, the register and the stock list are no longer sharing it with a form.

The heading of every screen shows the screen name and a one-line summary underneath — for example "2 waiting · 1 completed · Sun, 26 Jul".

At the bottom left it reminds you where the data lives and that a backup is taken each day the application is opened. Under that is who built the software and which version you are running:

Developed by Sivayaan Technologies Version 1.0.0.4

Read that version number out if you ever ring for help — it says which build of the software you have, which is the first thing anyone helping you needs to know. It is on screen whatever page you are on, so nobody has to go looking for it.

Screens clear themselves when the job is done

Whenever you finish something — save a patient, save a medicine, add a line to a bill, put stock on the shelf — the screen empties itself, **including the search box**, and the message underneath tells you what was saved.

This is deliberate and it matters. A form that still holds the last record is not ready for the next one: typing the next patient's name into it and pressing Save would change the person you just entered instead of adding anybody. The same at the counter — the medicine you just billed would still be selected, and pressing Add again would re-do the line you already have.

So after every save you start from a clean screen. If you want to look at what you just saved, search for it again; it is there.

For the forms that open over the screen, "clear" means the form has **gone**, taking everything typed into it. There is nothing left behind to be saved by accident, because the next one gets a fresh form.

Nothing carries over between deliveries either — not the supplier, not the bill number. Those two used to stay, on the reasoning that one delivery note covers many medicines. In use that was the wrong trade: a supplier left in the box is a supplier silently attached to the next delivery, and a wrong supplier on a batch is worse than a blank one.

2. Settings — set this up first

Everything here prints on your bills, receipts and prescriptions.

Twinkle
Children's Hospital
OPD & Pharmacy

OPD

Patients

Pharmacy counter

Medicines

Inventory

Reports

Settings

Data is stored on this PC. A backup is taken each day if it is opened.
Developed by Sriyasan Technologies
Version 1.0.0.4

Settings
Clinic / Pharmacy details printed on bills and prescriptions

Clinic / Pharmacy details

These appear on every printed bill and prescription. GSTIN and the drug licence number are required on a retail chemist's tax invoice.

Clinic / Pharmacy name

Twinkle Children's Hospital

Address

Phone

GSTIN

36ABCDE1234F1Z5

☒ Registered for GST

Bills print as TAX INVOICE with GSTIN, GST columns and a GST summary.

Drug licence no

Pharmacist

Consulting hours

Morning

10:00

to

13:00

Evening

16:00

to

20:00

The OPD screen can show the morning sitting (10:00-13:00), the evening one (16:00-20:00), or the full day. A visit booked outside both is only on the full day.

OPD queue layout

Tiles

Appearance

Light

Bill footer

Get well soon. Medicines once sold are not returnable.

Save Clinic / Pharmacy details

Clear

Saved. The OPD queue will use tiles, in the light theme.

Doctors

At least one doctor is needed before a visit can be booked.

Dr. A. Kumar

Name *

Speciality

Registration no

Default consultation fee

0

Save doctor

Clear

+ New doctor

Clinic / Pharmacy details (left card)

Control	What to enter
Clinic / Pharmacy name	Printed largest, at the top of every document
Address	One line, under the name
Phone	Shown beside the address
Registered for GST	Off by default. See below — this changes what your bills are
GSTIN	Your GST number. Only enabled when registered
Drug licence no	Your 20B/21B number. Required on a chemist's bill
Pharmacist	Printed at the foot of the bill
Consulting hours	When the doctor sits, morning and evening. See below
OPD queue layout	Tiles or Rows — see section 3
Appearance	Light or Dark . Changes the whole application straight away
Bill footer	Free text at the bottom of a bill, e.g. "Get well soon"
Save Clinic / Pharmacy details	Applies everything above. New documents use it immediately

Twinkle Children's Hospital

9

Consulting hours

Most clinics run two sittings with the afternoon off. Set the hours here and the OPD screen can show one sitting at a time:

Morning

10:00

to

13:00

Evening

16:00

to

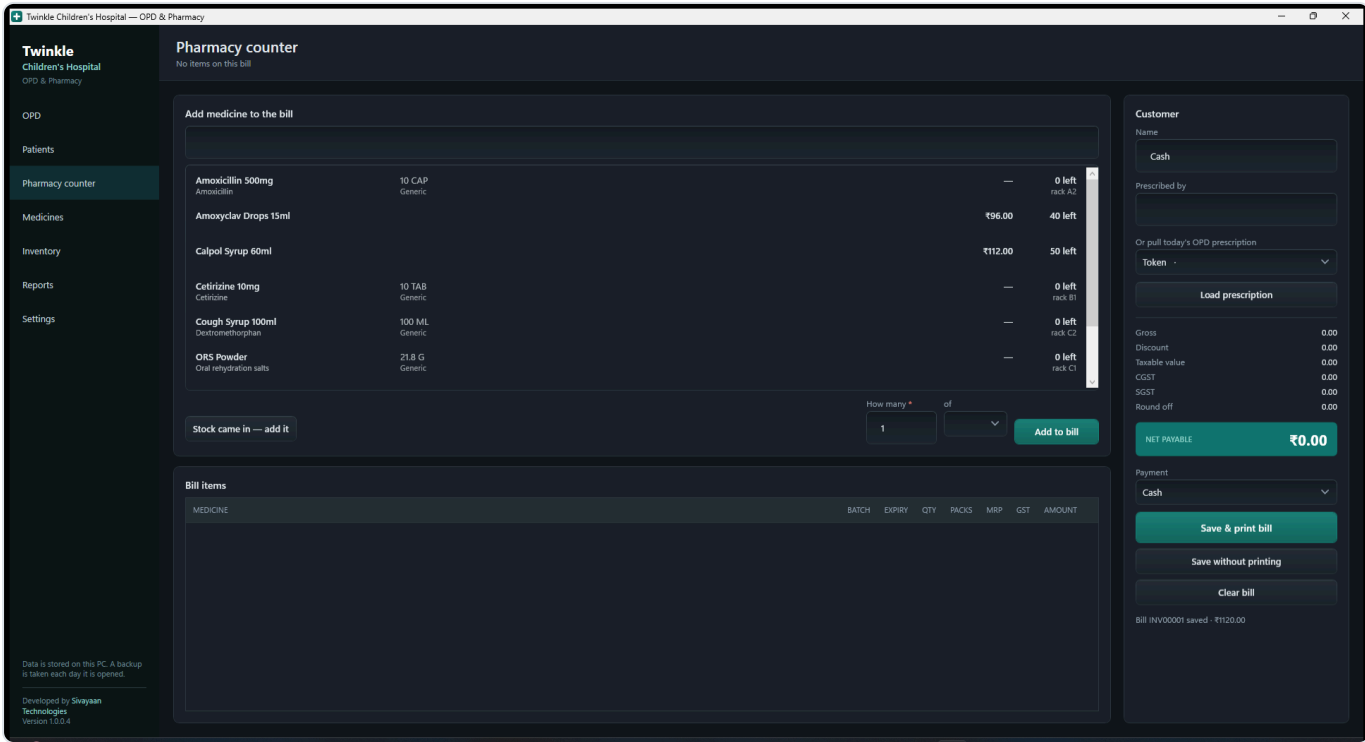
20:00

Use the 24-hour clock. The line underneath reads the four boxes back as a sentence, so a typo is caught before it is saved rather than the next time somebody wonders where the queue went. The end of a sitting is **exclusive** — a morning ending at 13:00 does not also claim the one o'clock patient.

These are only a filter on the OPD screen. Nothing stops you booking a visit outside them, and a visit booked outside both sittings still shows on **Full day** — see [section 3](#).

Appearance is worth trying if the counter sits under a bright window or, more often, in a dim corner in the evening. The change is immediate — you do not have to save or restart to see it — but **Save Clinic / Pharmacy details** is what makes it stick for next time.

Printed bills, receipts and prescriptions are always black on white whatever you choose here. Paper is paper.



Below the button it shows the **database file** and the **activity log** path, with an **Open log folder** button. You need those only if reporting a problem.

The application works with these blank — the GSTIN line simply does not print. Fill them in before you issue a real bill to a customer.

Registered for GST, or not

This one setting changes what your bills legally are.

	Registered off	Registered on
Bill is headed	INVOICE	TAX INVOICE
GSTIN printed	no	yes
GST charged	none	extracted from the MRP
GST column and summary	hidden	shown

It is **off by default on purpose**. Issuing a document headed "tax invoice", with a GSTIN on it, when you are not registered is a false statement — so switching it on has to be a deliberate act.

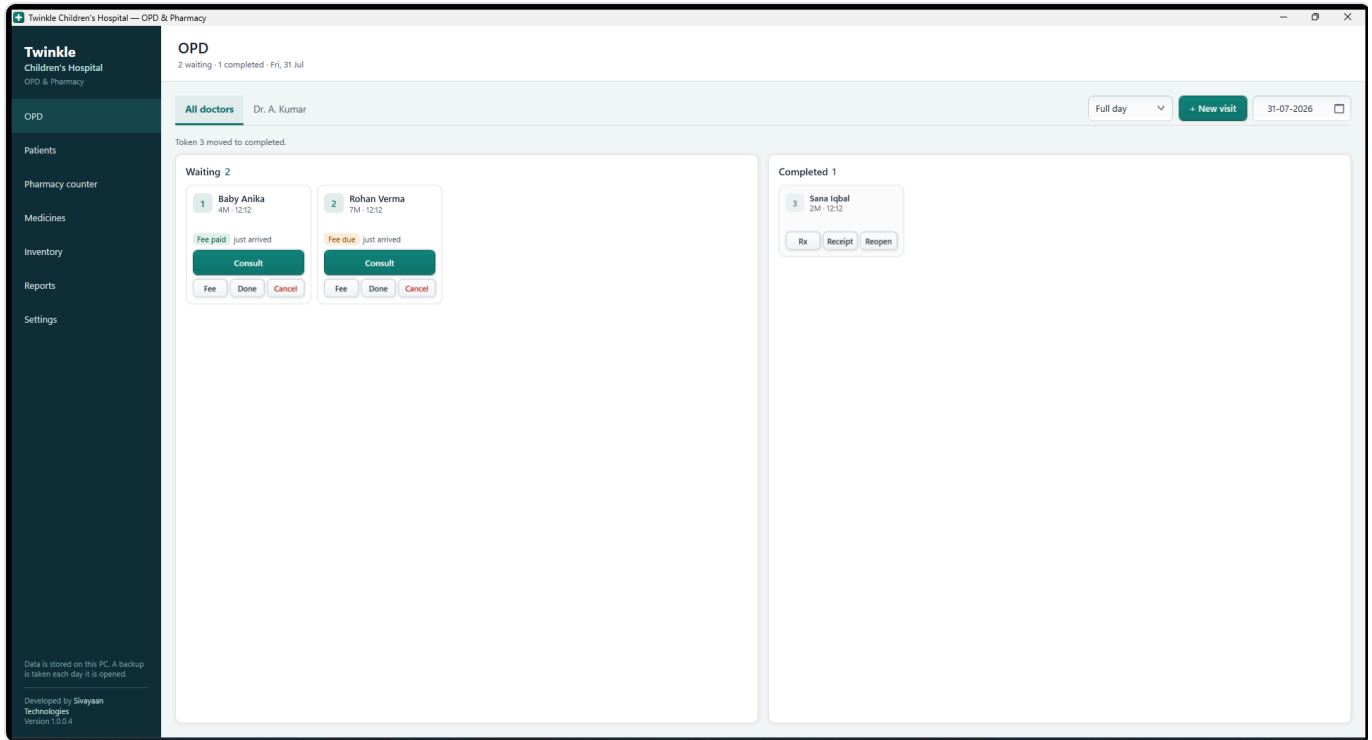
Turning it on later does not rewrite old bills. Each bill remembers what it was when it was issued, so a reprint always shows what the customer was given.

Doctors (right card)

Control	What it does
List	Every doctor. Click one to edit it
Name	Appears as an OPD tab and on the prescription
Speciality	Printed under the doctor's name on the prescription
Registration no	Printed on the prescription. Required on a real one
Default consultation fee	Fills in automatically when booking for this doctor
Save doctor	Saves the one being edited, then clears the form
+ New doctor	Clears the form to add another

At least one doctor is needed before any visit can be booked.

3. The OPD screen



Top row

Control	What it does
All doctors tab	Shows every patient in the clinic
Doctor tabs	One per doctor. Shows only their patients
Sitting	Full day , Morning or Evening . See below
+ New visit	Opens the booking form over the screen
Date	Which day you are looking at. Defaults to today

The doctor is a tab rather than a column on each patient, which is why the tiles stay small.

Morning, evening or the full day

Doctors sit mornings and evenings with the afternoon off, so "*who is left this evening*" is usually the real question. The picker narrows both columns to one sitting; the hours come from **Settings** → **Consulting hours**.

The heading then names the sitting and its hours:

3 waiting · 1 completed · Fri, 31 Jul · **Morning sitting, 10:00 to 13:00**

It tells you when it is hiding somebody. A visit booked at two in the afternoon belongs to neither sitting, so on **Evening** the heading reads:

0 waiting · 0 completed · Fri, 31 Jul · Evening sitting, 16:00 to 20:00 · **1 more today outside these hours**

Switch to **Full day** and everybody is there. Full day is how the screen starts, and it hides nobody — so if a patient seems to have vanished, that is the first thing to check.

The two columns

Waiting — everyone still to be seen. **Completed** — everyone finished.

A patient moves from one to the other and can be moved back.

What a waiting tile shows

On the tile	Meaning
Green number	Token number. What you call out
Name	The patient
4F · 08:55	Age, sex, and the time they were booked
Grey line	What they came in with
Fee paid / Fee due	Green if the consultation fee is taken, amber if not
just arrived / waiting 12m	How long they have been sitting there

Buttons on a waiting tile

Button	What it does
Consult	Opens the consultation window for this patient
Fee	Opens the fee form — see section 5
Done	Moves the tile to Completed without a consultation
Cancel	Cancels the visit. Asks first

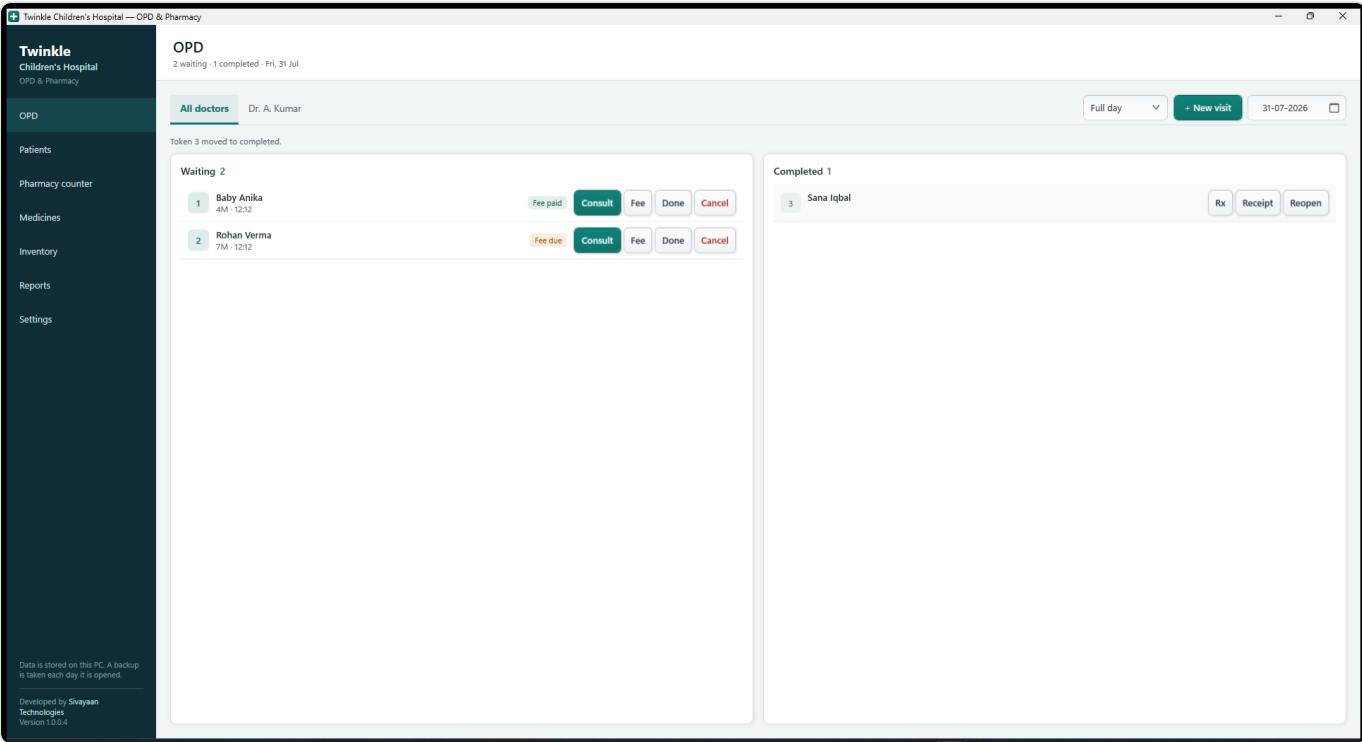
Buttons on a completed tile

Button	What it does
Rx	Prints the prescription. Says so if there isn't one
Receipt	Prints the fee receipt again, marked DUPLICATE

Button	What it does
Reopen	Moves the patient back to Waiting

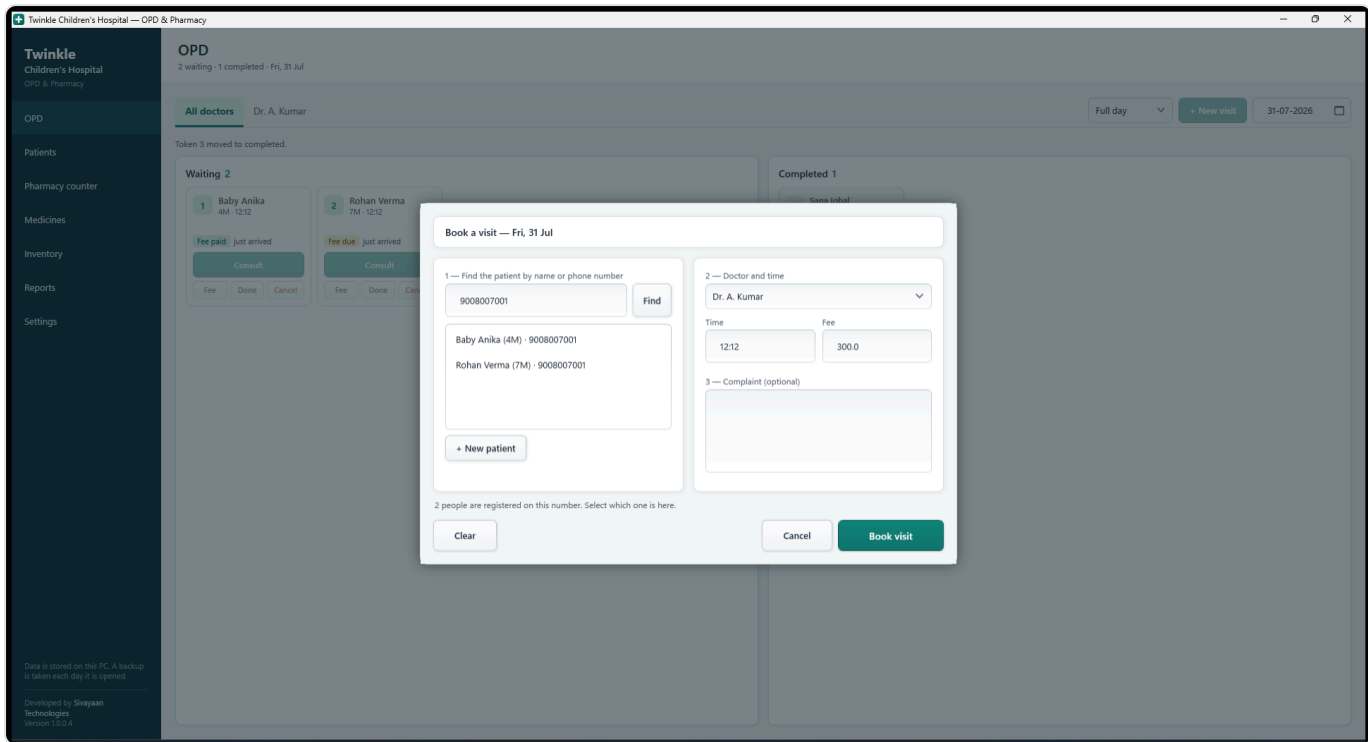
Choosing tiles or rows

Set in Settings. Tiles are easier to read across a room; rows fit more people on screen when the clinic is busy. **Everything works the same in either.**

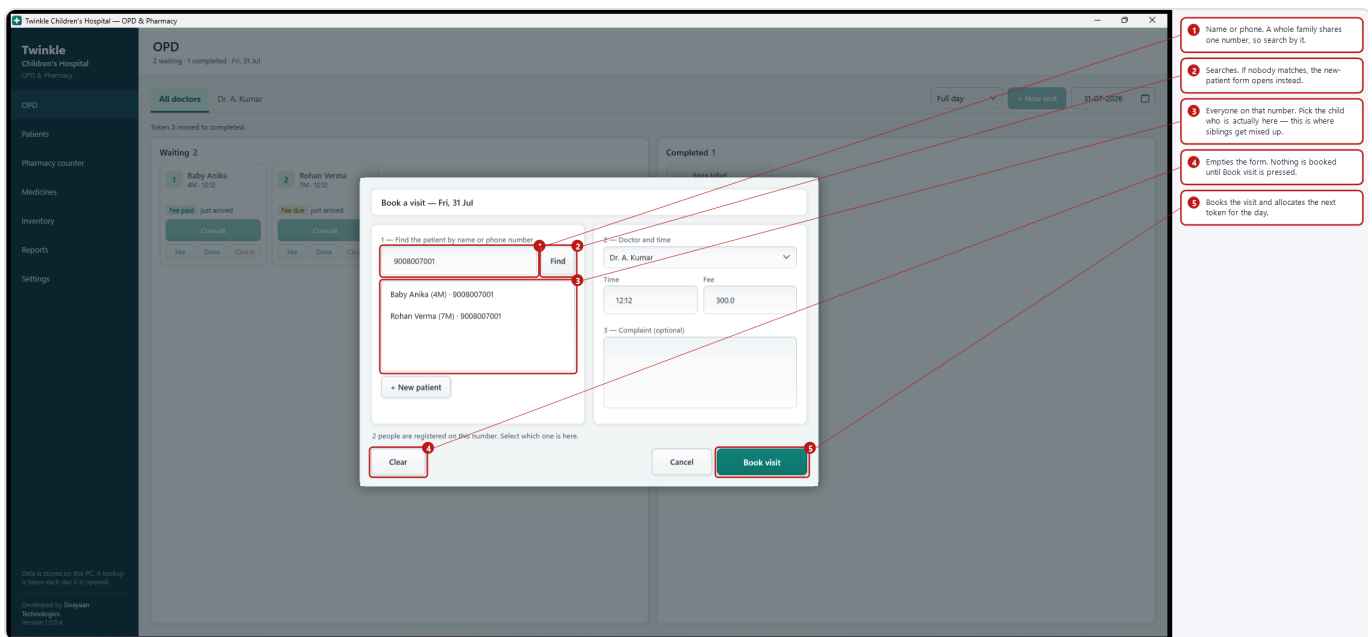


4. Booking a visit

Click **+ New visit**. The form opens over the screen, in three numbered steps — who is here on the left, when and why on the right.



Every control, ringed and explained:



Step 1 — find the patient

Control	What it does
Search box	Type a name or a phone number. Enter also works
Find	Runs the search
Results list	Everyone matching. Click the right one
+ New patient	Opens the form below to register someone new

If nobody matches, the new-patient form opens by itself with what you typed already filled in.

New patient field	Notes
Name	The only one that is required
Phone	The parent's number. Shared across siblings is normal
Age	In years
Sex	Male, Female or Other
Back to search	Returns to the list without adding anyone

Step 2 — doctor and time

Control	Notes
Doctor	Defaults to whichever tab you were on
Time	24-hour, e.g. 09:45 . Defaults to now
Fee	Defaults to that doctor's usual charge

Step 3 — complaint

Optional. Whatever you type shows on the queue tile and on the prescription.

Then **Book visit**. A token number is allocated automatically and the form closes. **Clear** empties the form without closing it; **Cancel** closes it and books nothing.

The payment method is no longer chosen here — it is asked for when you actually take the money, in [section 5](#).

One phone, several children

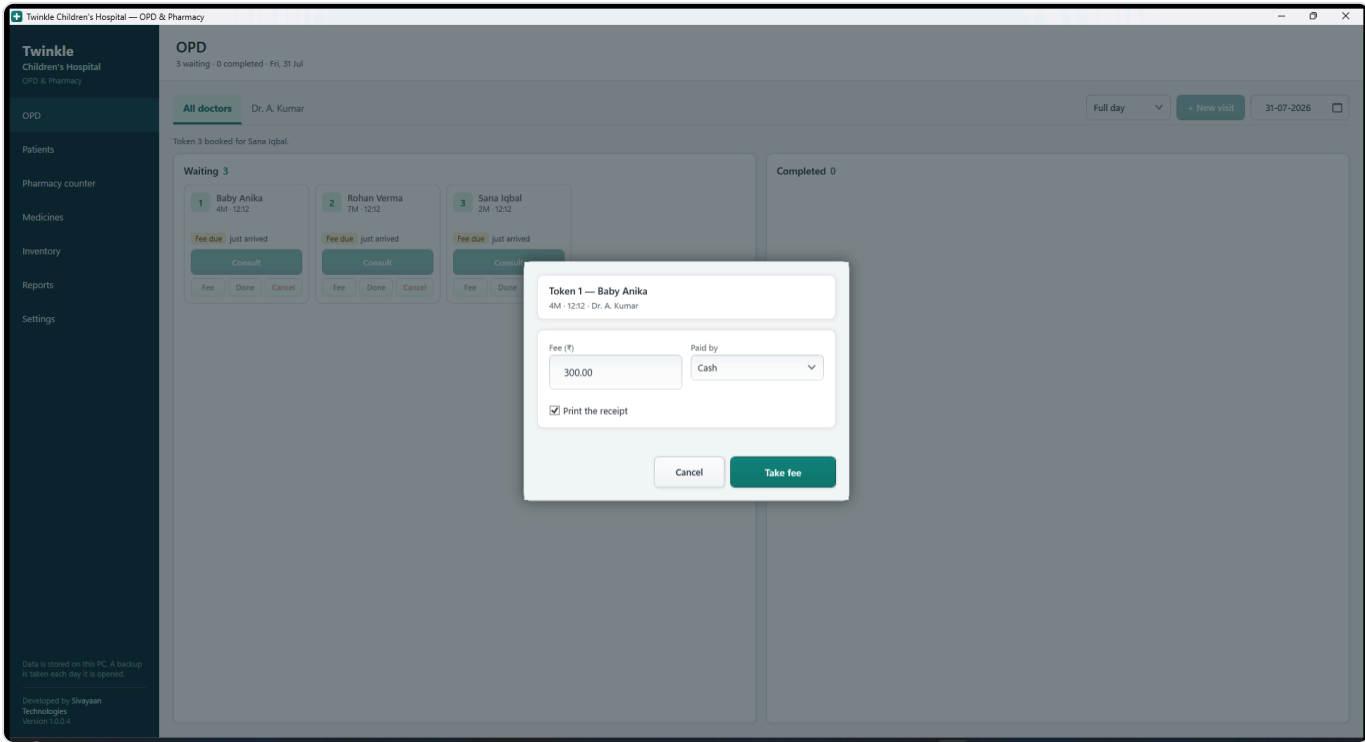
A parent's number covers the whole family. Typing it lists **every child** registered against it, and the message says so: "3 people are registered on this number. Select which one is here."

The number is matched on its digits, so 9008007001 , +91 90080 07001 and 90080 07001 all find the same family.

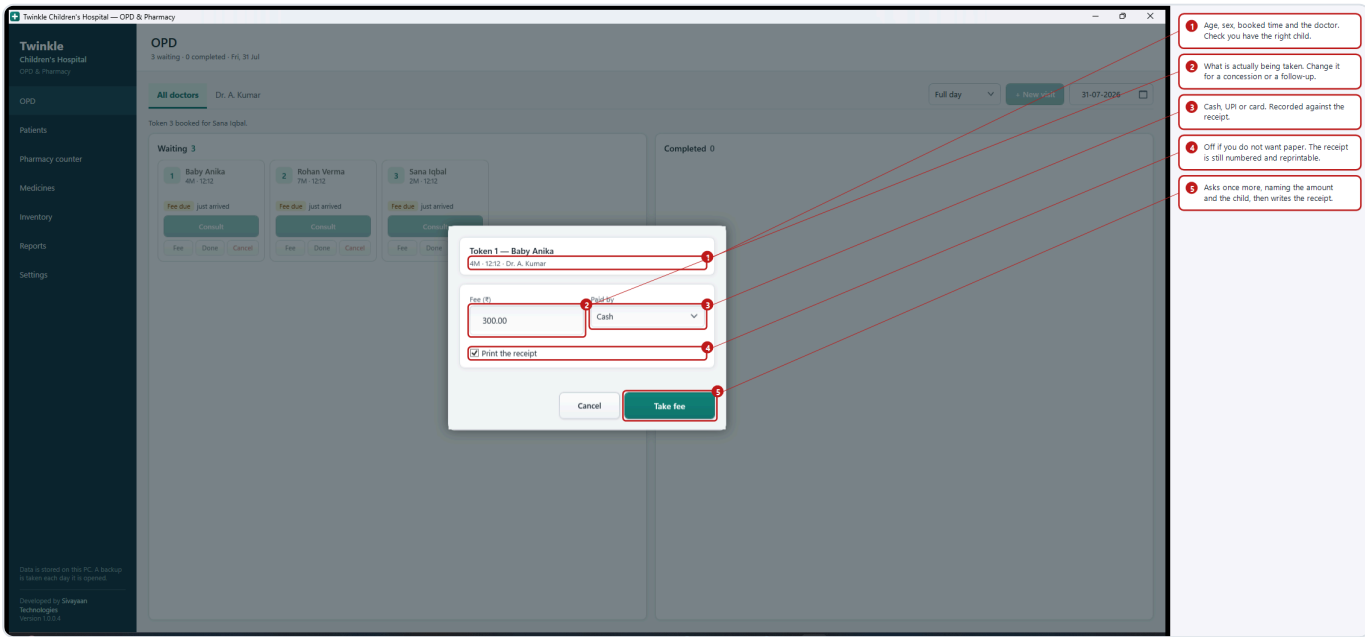
If you click **Book visit** without choosing one, the application **stops you** rather than quietly registering a fourth child who already exists.

5. Taking the consultation fee

Click **Fee** on a waiting tile. A small form opens over the screen.



Every control, ringed and explained:



- 1 Age, sex, booked time and the doctor. Check you have the right child.
- 2 What is actually being taken. Change it for a concession or a follow-up.
- 3 Cash, UPI or card. Recorded against the receipt.
- 4 Off if you do not want paper. The receipt is still numbered and reprintable.
- 5 Asks once more, naming the amount and the child, then writes the receipt.

Control	What it does
Heading	Token number and the patient's name
Line underneath	Age, sex, the booked time and the doctor
Fee (₹)	What you are actually taking. Filled in from the booking
Paid by	Cash, UPI or Card. Recorded against the receipt

Control	What it does
Print the receipt	On by default. Turn it off if you do not want paper
Cancel	Closes it. Nothing is taken and no receipt number is used
Take fee	Asks once more, then writes the receipt

Changing the amount

The fee starts at whatever was quoted when the visit was booked. Change it for a follow-up seen at half price, a family concession, or a rounding down. A note appears the moment it differs:

Booked at ₹300.00. This receipt will say ₹150.00.

That note is there because a concession is a decision and a mistyped digit is not, and on screen the two look identical.

The confirmation

Take fee does not take the fee. It asks:

Take ₹300.00 from Baby Anika by Cash?

Only **Yes** writes anything. This is the last gate before a receipt number is burnt, and it names the three things that get mixed up when two people are at the desk at once — the amount, the method and the child.

Afterwards the badge on the tile turns green to **Fee paid**, and the message under the doctor tabs gives the receipt number. Pressing **Fee** again does nothing; use **Receipt** on the completed tile, or the patient's own record, to print it a second time.

Why it asks at all. A receipt is numbered and dated the moment it is written. There is no way to un-write one — a fee taken wrongly has to be reversed on paper. Pressing Fee used to take the money immediately, at whatever payment method a box at the top of the screen had been left on, and go straight to a print preview. Now nothing happens until you have seen the amount, the method and the name.

6. The consultation

Click **Consult** on a tile.

Every control, ringed and explained:

The heading shows the token, the patient, their age and sex, and the doctor.

Left — clinical notes

Control	Notes
Weight kg, BP, Temp °F	Vitals. All optional
Complaint	Carried over from booking; edit freely

Control	Notes
Diagnosis	Printed in bold on the prescription
Advice / notes	Printed under the medicines
Fee	Can be changed here
Review on	Follow-up date. Printed as "Review on 01 Aug 2026"

Right — the prescription

Fill the form at the top and press **Add**. Each medicine then appears in the list below.

Control	Notes
Medicine	Type two letters or more to search our pharmacy. Matches appear underneath — click one to use it
Dose	e.g. 5 ml , 1 tab
Morning · Afternoon · Night	Three lists: 0 , 1/4 , 1/2 , 1 , 2 . Pick how many at each time of day
Days	Length of the course
	<i>Nothing is filled in for you — a dose is the doctor's decision, not the software's</i>
Qty (units)	Individual tablets. Worked out for you — change it if you want
Grey line under the fields	What the course comes to, in tablets and in strips
Instructions	e.g. "after food". Printed under the line
Add	Adds the medicine to the list
✕ in the list	Removes that medicine

The three lists replace the old typed frequency box. 1-0-1 was quick for anyone who already knew the notation and a guess for everyone else; picking from three lists cannot be mistyped, and the prescription still prints in the familiar 1-0-1 form.

Frequency and days are **kept after adding**, because a prescription usually repeats the same course — only the medicine changes.

Quantity is always in individual units

You pick **1**, **0**, **1** for **3** days and it fills in **6** — six tablets, not six strips. The line underneath shows what the pharmacy will hand over, e.g. "6 units · 1 × 10 TAB minus 4", so there is never any doubt about whether a number means tablets or strips.

SOS and **PRN** have no fixed daily dose, so nothing is filled in and you type the quantity yourself.

Prescribing something the clinic does not stock

Type the name and simply **do not pick anything from the list**. The line reads "Not in our pharmacy — it will be written on the prescription only", and it is added exactly as typed. The parent buys it from an outside chemist.

It is never added to our medicine records. A name typed on a prescription does not create a medicine, a batch, or a price — our catalogue only ever grows when stock is received.

A medicine you **do** pick from the list shows "In our pharmacy · 60 in stock" and can be pulled straight onto a bill at the counter, with the quantity already correct. One prescription can mix the two freely.

Bottom buttons

Button	What it does
Close	Leaves the consultation. Asks first if anything is unsaved
Save	Keeps everything. Patient stays in Waiting
Print prescription	Saves, then opens the print preview
Save & complete	Saves and moves the tile to Completed

The **Esc** key does the same as **Close**. While the consultation is open the rest of the app is greyed out and waits — you cannot wander off to another screen with a half-written prescription behind you.

7. Medicines — the catalogue

What each medicine **is**. Set up once per medicine; stock is the next screen.

Twinkle Children's Hospital — OPD & Pharmacy

Medicines
8 medicine(s) in the catalogue

Search Edit + New medicine

MEDICINE	PACK	MAKER	RACK	GST	SCH	PER PACK	STOCK
Amoxicillin 500mg Amoxicillin	10 CAP	Generic	A2	12%	H	10	Out
Amoxycylav Drops 15ml				12%	—	1	40
Calpol Syrup 60ml				12%	—	1	60
Cetirizine 10mg Cetirizine	10 TAB	Generic	B1	12%	—	10	Out
Cough Syrup 100ml Dextromethorphan	100 ML	Generic	C2	12%	—	1	Out
ORS Powder Oral rehydration salts	21.8 G	Generic	C1	5%	—	1	Out
Pantoprazole 40mg Pantoprazole	15 TAB	Generic	B2	12%	—	15	Out
Paracetamol 500mg Paracetamol	15 TAB	Generic	A1	12%	—	15	Out

Data is stored on this PC. A backup is taken each day it is opened.

Developed by **Sreyan Technologies**
Version 1.0.0.4

Amoxycylav Drops 15ml saved.

The catalogue

The grid has the whole window: medicine, pack, maker, rack, GST %, schedule, units per pack and stock on hand. Above it sit the search box, **Search**, **Edit** and **+ New medicine**.

Search matches the **brand name**, the **drug name**, the manufacturer and the rack. Typing `paracetamol` finds Calpol; typing `calpol` finds it too.

To change a medicine, select the row and click **Edit** — or just **double-click the row**, which does the same thing.

The medicine form

Opens over the screen, in three columns: what it is called, what a pack is, and what it costs and where it lives.

The screenshot shows a web application for 'Twinkle Children's Hospital — OPD & Pharmacy'. On the left is a sidebar with navigation links: OPD, Patients, Pharmacy counter, Medicines (highlighted), Inventory, Reports, and Settings. The main area is titled 'Medicines' and shows a list of medicines on the left and a table of stock on the right. A modal form titled 'Medicine — Calpol Syrup 60ml' is open in the center. The form has the following fields: Brand name (filled with 'Calpol Syrup 60ml'), Manufacturer (empty), HSN (filled with '3004'), GST % (filled with '12.0'), Drug / generic name (empty), Pack size (empty), Schedule (dropdown menu with 'None' selected), Rack (empty), Reorder level (filled with '0'), Composition (empty), Units in one pack (filled with '1'), Storage (empty), Sold as (dropdown menu with 'Bottle' selected), and checkboxes for 'Active' (checked) and 'Sell loose units' (checked). At the bottom of the form are 'Cancel' and 'Save medicine' buttons. The background table shows various medicines with columns for Pack, Maker, Rack, GST, Sch, Per Pack, and Stock.

Field	Notes
Brand name	What is printed on the pack — <i>Calpol</i> . The only required field
Drug / generic name	The drug itself — <i>Paracetamol 250mg</i> . Searched as well
Manufacturer	Company name
Pack size	As printed, e.g. 10 TAB , 60ML
HSN	Tax code. 3004 covers most formulations
GST %	Usually 5 or 12
Schedule	None , H , H1 , X . H1 sales are recorded in a register
Rack	Where it sits on the shelf. Searchable
Reorder level	Below this it appears in the Low stock report
Units per pack	10 for a ten-tablet strip. 1 for a syrup bottle
Sell loose units	Ticked, part of a strip can be sold. Unticked, the counter insists on whole packs
Active	Uncheck to hide it from the counter
Save medicine	Saves it and closes the form. The catalogue clears — search box included — for the next medicine
Cancel	Closes without saving. Nothing already saved is changed

There is no Clear button, because there is nothing to clear: the form holds one medicine and Cancel takes it away. The next one gets a fresh form.

Units per pack is what makes loose sale work

Set it to 10 for a strip of ten tablets and stock is counted in **tablets**, so a customer can buy five. Leave it at 1 for a syrup bottle — half a bottle is not a thing you can sell.

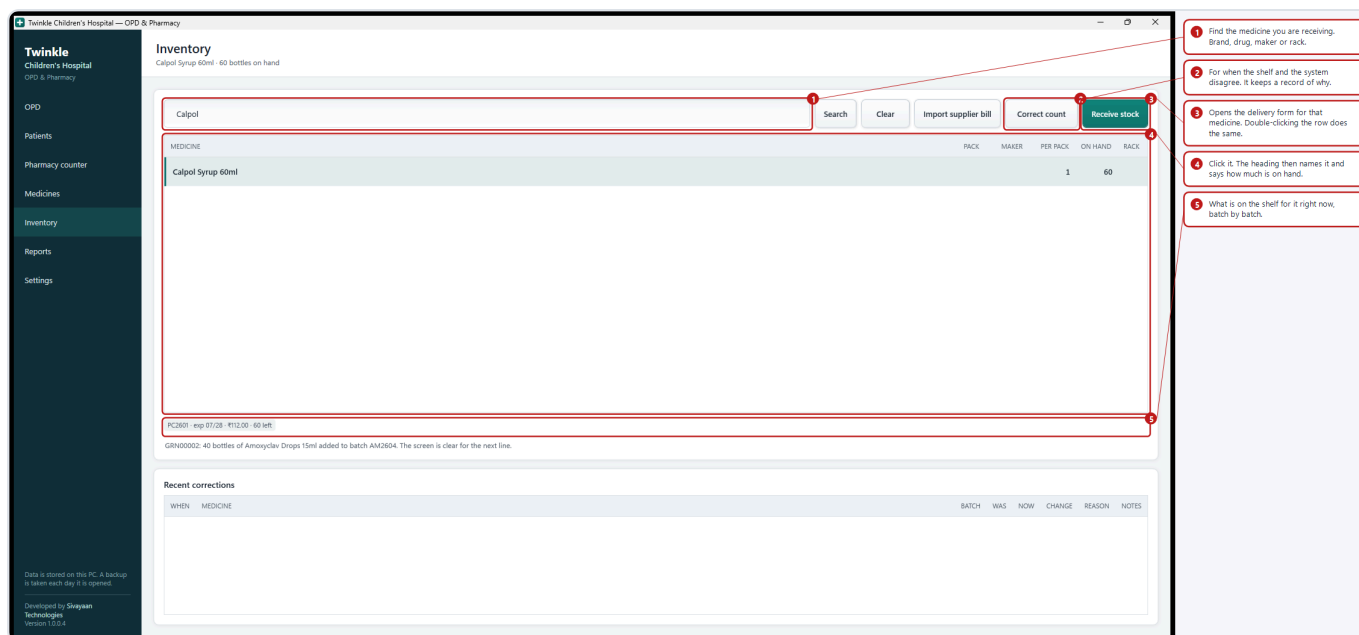
8. Inventory — what is on the shelf

Everything to do with stock: receiving it, seeing the batches, correcting a count. Nothing here changes what a medicine is — that is the Medicines screen.

The screenshot shows the 'Inventory' screen of the Twinkle Children's Hospital software. The interface is divided into a left sidebar and a main content area. The sidebar contains navigation links: OPD, Patients, Pharmacy counter, Medicines, Inventory (highlighted), Reports, and Settings. The main content area has a header 'Inventory' with a subtitle 'Calpol Syrup 60ml - 60 bottles on hand'. Below this is a search bar with 'Calpol' entered, and buttons for 'Search', 'Clear', 'Import supplier bill', 'Correct count', and 'Receive stock'. A table lists the inventory items with columns: MEDICINE, PACK, MAKER, PER PACK, ON HAND, and RACK. The table shows 'Calpol Syrup 60ml' with 1 pack and 60 bottles on hand. Below the table, there are two status messages: 'PC2601 - exp 07/28 - ₹112.00 - 60 left' and 'GRN00002: 40 bottles of Amoxycylav Drops 15ml added to batch AM2604. The screen is clear for the next line.' At the bottom, there is a 'Recent corrections' section with a table with columns: WHEN, MEDICINE, BATCH, WAS, NOW, CHANGE, REASON, and NOTES. The footer of the sidebar contains a note: 'Data is stored on this PC. A backup is taken each day it is opened.' and 'Developed by Sruyan Technologies Version 1.0.0.4'.

MEDICINE	PACK	MAKER	PER PACK	ON HAND	RACK
Calpol Syrup 60ml	1			60	

Every control, ringed and explained:



Finding the medicine

Type any part of the brand name, drug name, maker or rack and click **Search**, then click the row. The heading at the top of the screen then names the medicine and how much is on hand, so you always know what you are about to change.

Underneath the grid, **the batches on the shelf** for the selected medicine appear as small chips — batch number, expiry, MRP and how much is left.

Along the top: **Search**, **Clear**, **Import supplier bill** (see [section 9](#)), **Correct count** and **Receive stock**. The last two need a medicine selected first.

At the foot of the screen, **Recent corrections** lists every count that has been put right — when, what, was, now, the change, the reason and any note.

Receive stock

Select the medicine, then click **Receive stock**. A form opens over the screen, headed with the medicine's name and what is already on the shelf.

This is the only way stock enters, and it always creates a batch.

Twinkle Children's Hospital — OPD & Pharmacy

Inventory
Calpol Syrup 60ml - 60 bottles on hand

Calpol

Search Clear Import supplier bill Correct count **Receive stock**

MEDICINE

PACK MAKER PER PACK ON HAND RACK

Calpol Syrup 60ml

1 60

Receive stock — Calpol Syrup 60ml
60 bottles on hand - sold as a single bottle

Batch no * Expiry * 31-07-2028

Packs * Free packs

0 0

Rate per pack MRP per pack *

0 0

Supplier Supplier bill no

Cancel **Add stock**

PC3601 - exp 07/28 - ₹112.00 - 60 left
GRN00002: 40 bottles of Amoxycylav Drops 15ml added to batch AM2604.

Recent corrections

WHEN MEDICINE BATCH WAS NOW CHANGE REASON NOTES

Data is stored on this PC. A backup is taken each day it is opened.

Developed by **Sreyan Technologies**
Version 1.0.0.4

Every control, ringed and explained:

Twinkle Children's Hospital — OPD & Pharmacy

Inventory
Calpol Syrup 60ml - 60 bottles on hand

Calpol

Search Clear Import supplier bill Correct count **Receive stock**

MEDICINE

PACK MAKER PER PACK ON HAND RACK

Calpol Syrup 60ml

1 60

Receive stock — Calpol Syrup 60ml
60 bottles on hand - sold as a single bottle

Batch no * Expiry * 31-07-2028

Packs * Free packs

0 0

Rate per pack MRP per pack *

0 0

Supplier Supplier bill no

Cancel **Add stock**

PC3601 - exp 07/28 - ₹112.00 - 60 left
GRN00002: 40 bottles of Amoxycylav Drops 15ml added to batch AM2604.

Recent corrections

WHEN MEDICINE BATCH WAS NOW CHANGE REASON NOTES

Data is stored on this PC. A backup is taken each day it is opened.

Developed by **Sreyan Technologies**
Version 1.0.0.4

- 1 Printed on the pack. Required — it has to appear on the bill by law.
- 2 The pack is good until the **END** of that month.
- 3 Scheme quantity: the "+1" in 10+1. Goes on the shelf, costs nothing.
- 4 How many **PACKS** arrived — strips, boxes or bottles. Not tablets.
- 5 The price printed on the pack. The counter prices everything from this.
- 6 Adds to the shelf. It never replaces what is already there.

Field	Notes
Batch no	Printed on the pack. Required — it goes on the bill by law
Expiry	The pack is good until the end of that month
Packs	How many packs arrived — strips, boxes, bottles
Free packs	Scheme quantity, the "+1" in 10+1. Adds to stock, costs nothing
Rate per pack	What the hospital paid per pack

Field	Notes
MRP per pack	The price printed on the pack. The counter prices from this
Supplier, Supplier bill no	For your records, and what the reconciliation report matches against
Add stock	Adds it and closes the form, ready for the next line
Cancel	Closes it. Nothing goes on the shelf

Packs in, units out

Above the buttons a grey line reads back what you have entered: *"20 pack(s) × 15 = 300 tablets onto the shelf"*. You count deliveries in strips, the counter sells tablets, and this is the one place the two meet — so it says so out loud rather than leaving you to trust it.

Everything goes when the form closes — the supplier and bill number included — so a price, an expiry or a supplier from the last delivery can never be carried onto the wrong batch, and a second batch can never be received against a medicine you had forgotten was still selected.

Double-clicking a row in the grid opens this form too.

Adding stock always adds. Receiving the same batch number again increases what is on the shelf. It never replaces it.

Correct count

For when the shelf and the screen disagree — breakage, a miscount, or something keyed in wrongly. Select the medicine and click **Correct count**.

Every control, ringed and explained:

Control	Notes
Batch	Which batch is wrong. Shows expiry, MRP and current count. Chosen for you when there is only one
True count	What is actually on the shelf, in units. Starts at what the system believes
Reason	Recount , Breakage , Expired , Lost , Entry error , Other
Notes	Free text — worth writing for anything unusual
Correct count	Applies it and closes the form

Step 1 — choose the profile and the file

Control	What it does
Supplier profile	Which supplier's format this is. Each knows that supplier's date and expiry style
File	The path. Use Browse
Supplier name	Type this — the file does not contain it
Browse...	Pick the file. Reads it immediately
Read file	Re-reads it after changing the profile

Step 2 — check what it will do

The summary line reads, for example: "Bill SW02236 dated 04 Jul 2026 · 9 line(s) · 9 new medicine(s) · 1042 unit(s) · net ₹15334.00"

Column	Meaning
STATUS	Matched — found in your catalogue · Check — a likely match, confirm it · New medicine — will be created
MEDICINE, PACK, BATCH, EXPIRY	As the supplier sent them
PER PACK	Editable. Units in one pack
QTY, FREE	Packs billed, and free packs
UNITS	What will land on the shelf: (qty + free) × per pack
RATE, MRP, GST	Cost, printed price, tax rate

What the file says below lists everything worth knowing — the bill date it read, MRP changes, anything it could not understand.

Check the PER PACK column

The file rarely states how many are in a pack. **30s** is read as 30 gummies. **60ML** is one bottle and stays **1** — a syrup is not sixty sellable units.

For strips, **type the real number** before importing. Correct it once and the medicine remembers it.

Step 3 — import

Import writes everything in one go. **Close** abandons it — nothing is written until you click Import.

Afterwards it reports: "Imported as GRN00003: 9 line(s), 9 new medicine(s), 1042 unit(s) added to stock."

What the import guarantees

- **Stock is added**, never replaced. 12 counted by hand plus 5 on the bill is 17
- **The same bill cannot be imported twice**. It is refused by bill number
- **Nothing is written if anything fails**. All of it, or none
- **The supplier's product codes are remembered**, so their next bill matches by itself

10. The pharmacy counter

Three steps per line: **find the medicine, set the quantity, add**.

Every control, ringed and explained:

The screenshot shows the 'Pharmacy counter' interface. On the left is a sidebar with navigation links: Twinkle Children's Hospital, OPD & Pharmacy, OPD, Patients, Pharmacy counter (selected), Medicines, Inventory, Reports, and Settings. The main area is titled 'Pharmacy counter' and shows a list of medicines with columns for name, generic name, quantity, price, and stock. A search bar is at the top. A 'Stock came in — add it' button is highlighted with a red box and a callout. A 'How many' input field is also highlighted. The 'Add to bill' button is highlighted. The 'Bill items' section shows a table with columns for medicine, batch, expiry, qty, packs, MRP, GST, and AI. The 'Customer' section shows a form for Name, Prescribed by, and Token. The 'NET PAYABLE' is ₹1,120.00. The 'Save & print bill' button is highlighted. The 'Save without printing' and 'Clear bill' buttons are also visible. The bottom of the screen shows a status bar with 'Data is stored on this PC. A backup is taken each day if it is opened.' and 'Developed by Shreyas Technologies Version 1.0.0.4'.

Callouts 1-8 explain the controls:

- Type any part of the brand, drug, maker or rack. It filters as you type — no button, any case.
- What is on the shelf. Price per unit and how many left. Click the one you want.
- How many. Nine tablets is 9 — never the number of strips.
- What that number counts. Tablets, or strips of 10. Remembered per medicine.
- The medicine is in the shop but the screen says none? Put it on the shelf without leaving the bill.
- What the customer pays. GST is already inside the MRP.
- The bill. Only QTY can be changed. The price comes from the batch and cannot be edited.
- Saves the bill, deducts the stock, and opens the print preview.

	Control	What it is for
1	Search	Any part of the brand, drug, maker or rack. Filters as you type, in any case
2	Results	What is on the shelf: price per unit, how many left, which rack
3	How many	The number. Nine tablets is 9
4	of	What that number counts — <i>tablets</i> or <i>strips of 10</i> . Remembered per medicine
5	Stock came in — add it	Puts stock on the shelf without leaving the bill
6	Bill items	Only QTY can be changed. The price comes from the batch
7	Net payable	What the customer pays. GST is already inside the MRP
8	Save & print bill	Saves, deducts the stock, opens the preview

Add medicine to the bill

Control	What it does
Medicine	Type part of the name. Enter searches
Find	Runs the search
Results list	Shows pack, maker, rack and stock. Click one
Batch	The nearest-expiry batch with stock is chosen for you
Qty (units)	Tablets, not strips. 5 sells five out of a strip
Add to bill	Adds the line, then empties the search and lets go of the medicine. The bill is untouched — type the next medicine's name to carry on
Stock came in — add it	Puts stock on the shelf from here. See below

The medicine is in the shop but the screen says none

It happens: a delivery arrived and nobody entered it, or it was bought in as a one-off. **Do not send the patient away and do not leave the bill.**

Click **Stock came in — add it**.

+

Add stock

×

Calpol Syrup 60ml

60 bottles on hand

Packs on the shelf *

MRP per pack *

Rate paid

5

112.0

80.64

One pack is one bottle.

5 bottles onto the shelf

Batch no (if you have it)

Expiry

31-07-2028

⚠ This goes on the shelf now so the patient can be served. It has no supplier

Cancel

Add to shelf

Field	Notes
Packs on the shelf	How many strips, boxes or bottles you are putting in
MRP per pack	Required — nothing can be priced without it. Filled in from the last time this medicine came in
Rate paid	Leave it if you do not know. It can be filled in when the bill arrives
Batch no	Enter it if it is on the pack. Leave it blank and one is allocated for you , starting CTR-
Expiry	Defaults to two years out. Change it if you know better
Add to shelf	Puts it in. You come straight back to the bill

The grey line reads back what you have entered — "5 pack(s) × 10 = 50 tablets onto the shelf" — before anything is committed.

What this costs you, and why it is worth it

Stock added this way has **no supplier bill behind it**, so total purchases will not tie out against total sales until that bill turns up. That is a deliberate trade: a counter that stops to do paperwork with a patient waiting is a counter nobody uses.

Nothing is hidden to buy that. Every entry is a proper goods-inward document with the quantity, the MRP, the date and who keyed it, and every one of them is listed under **Reports** → **Stock to reconcile** until it is matched to the real bill. You can square the books whenever you like — a week later, a month later — because nothing was done quietly.

Bill items

Column	Notes
MEDICINE, BATCH, EXPIRY	What is being handed over
QTY	Base units. Editable
PACKS	Reads back as 2 × 10 TAB + 3
MRP, DISC %	Editable
GST %, AMOUNT	Calculated
×	Removes the line

Customer (right)

Control	Notes
Name	Defaults to Cash . A walk-in needs no patient record
Prescribed by	Doctor's name. Required on the bill for a Schedule H1 drug
Or pull today's OPD prescription	Pick a patient seen today
Load prescription	Puts every prescribed medicine that is in stock on the bill

Totals

Gross	what the MRP comes to
Discount	anything taken off
Taxable value	the MRP with GST taken back out of it
CGST + SGST	the tax, split in half
Round off	to the nearest rupee
NET PAYABLE	what the customer hands over

Control	What it does
Payment	Cash, UPI or Card
Save & print bill	Saves and opens the preview
Save without printing	Saves only
Clear bill	Empties the counter without saving

Every bill is settled in full

There is no credit and no part payment — by design, because the clinic does not offer them. A bill is paid before it is saved, so nothing is ever outstanding and there is no balance to chase.

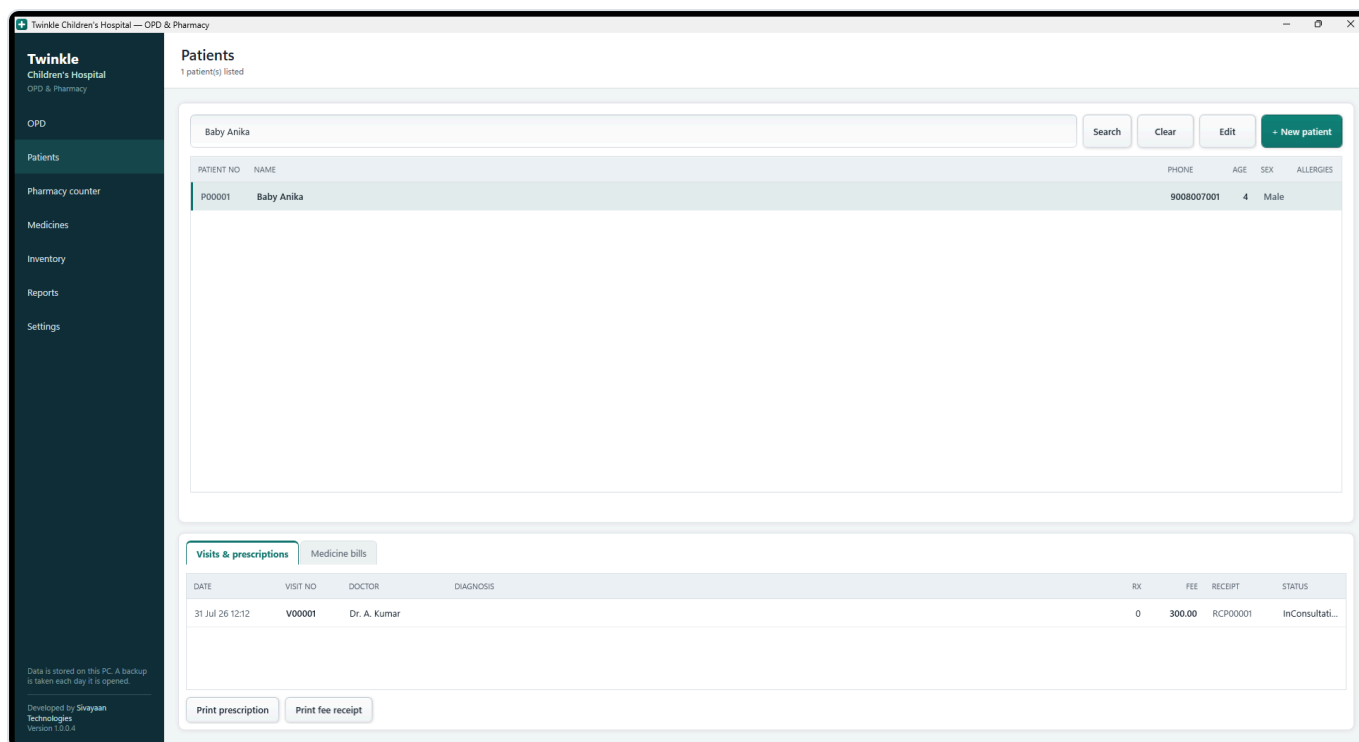
MRP already includes GST

Tax is never added on top. Ten strips at ₹112 MRP come to **exactly ₹1,120** — of which ₹1,000 is the taxable value and ₹120 is GST.

A whole strip always costs what is printed on it

Five tablets from a ₹112 strip of ten cost ₹56. But a **full** strip costs ₹112, not ten times the rounded per-tablet price. This matters where the price does not divide evenly: ₹87.50 across fifteen tablets is ₹5.83 each, and the full strip still costs ₹87.50.

11. Patients



Search by **name, patient number or phone**. A phone number lists the whole family.

The register

Patient no, name, phone, age, sex and allergies, across the whole window. Click a row to select it — the history underneath fills in with that patient's visits and bills.

Along the top: **Search, Clear, Edit** and **+ New patient**.

The patient form

+ New patient registers someone without booking a visit. **Edit** opens the selected patient — as does **double-clicking the row**.

Twinkle Children's Hospital — OPD & Pharmacy

Patients
1 patient(s) listed

Search Clear Edit + New patient

PATIENT NO	NAME	PHONE	AGE	SEX	ALLERGIES
P00001	Baby Anika	9008007001	4	Male	

Baby Anika P00001

Name * Address

Phone Allergies

Age Sex

4 Male

Remove Cancel Save patient

Visits & prescriptions Medicine bills

DATE	VISIT NO	DOCTOR	DIAGNOSIS	RX	FEE	RECEIPT	STATUS
31 Jul 26 12:12	V00001	Dr. A. Kumar		0	300.00	RCP00001	InConsultati...

Print prescription Print fee receipt

Data is stored on this PC. A backup is taken each day it is opened.

Developed by Shreyan Technologies Version 1.0.0.4

Every control, ringed and explained:

Twinkle Children's Hospital — OPD & Pharmacy

Patients
1 patient(s) listed

Search Clear Edit + New patient

PATIENT NO	NAME	PHONE	AGE	SEX	ALLERGIES
P00001	Baby Anika	9008007001	4	Male	

Baby Anika P00001

Name * Address

Phone Allergies

Age Sex

4 Male

Remove Cancel Save patient

Visits & prescriptions Medicine bills

DATE	VISIT NO	DOCTOR	DIAGNOSIS	RX	FEE	RECEIPT	STATUS
31 Jul 26 12:12	V00001	Dr. A. Kumar		0	300.00	RCP00001	InConsultati...

Print prescription Print fee receipt

Data is stored on this PC. A backup is taken each day it is opened.

Developed by Shreyan Technologies Version 1.0.0.4

- 1 The only field that must be filled in.
- 2 A whole family shares one number, and that is fine.
- 3 Shown against every prescription this child is written.
- 4 Only for somebody already on the register, and it asks first.
- 5 Saves and closes. The register clears for the next patient.

Field	Notes
Patient no	Top right. Allocated on save, e.g. P00012
Name	The only field that is required
Phone	The parent's number. Shared across siblings is normal
Age, Sex	
Address, Allergies	Optional. Allergies show against every prescription

Field	Notes
Save patient	Saves and closes. The register clears — search box included — for the next patient
Cancel	Closes without saving. Nothing already saved is changed
Remove	Only for somebody already registered, and it asks first. Refused if they have visits on record

The register clearing after Save is deliberate. This screen used to keep the patient loaded in a column on the right, and typing the next child's name over it and pressing Save changed the child you had just registered instead of adding the new one — the first child left the register without a trace. Each patient now gets a form of its own. To carry on editing the same patient, search for them again and click Edit.

History

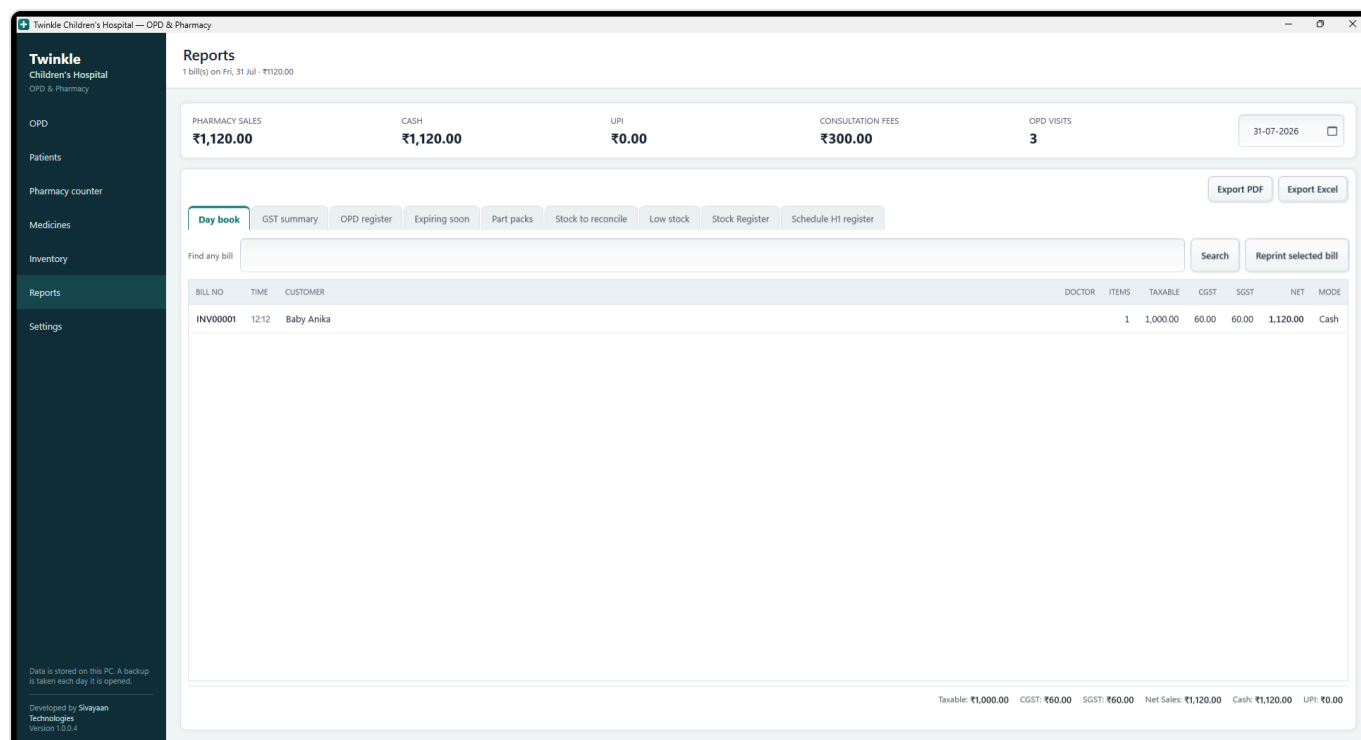
Visits & prescriptions — every visit ever, with diagnosis, fee and receipt number. Select one, then:

- **Print prescription** — prints it however long ago it was
- **Print fee receipt** — prints the receipt again, marked DUPLICATE

Medicine bills — every bill for this patient, with **Print bill**.

This is where you go when someone returns weeks later having lost a receipt.

12. Reports



Across the top for the chosen date: pharmacy sales, cash, UPI, consultation fees collected, and OPD visits.

Tab	What it is for
Day book	Every bill for the day. Find any bill searches every date by bill number or customer name. Reprint selected bill prints it again
GST summary	Taxable value, CGST and SGST grouped by rate — what a return needs
OPD register	Every visit, diagnosis, fee, and whether it was paid
Expiring soon	Batches within 90 days of expiry. Return these to the distributor
Low stock	Anything at or below its reorder level
Stock to reconcile	Everything put on the shelf at the counter with no supplier bill behind it. See below
Schedule H1 register	Statutory record of H1 sales. Keep for three years

Stock to reconcile

When the counter adds stock to serve a patient, there is no supplier bill behind it yet. Until there is, purchases will not tie out against sales — so every one of those entries is listed here with the date, medicine, batch, quantity on hand, MRP and the rate paid.

Work through it whenever it suits you. For each row, find the supplier bill it belongs to and check the quantity and the rate against it. A batch number starting **CTR-** was allocated by the system rather than read off the pack, so those are the ones worth checking against the real batch number.

The list is not a warning and does not have to be empty. It is a worklist.

13. Printing

Everything previews before any paper moves.

Print preview — Receipt RCP00001 (duplicate)

Twinkle

CHILDREN'S HOSPITAL

+91 81428 30194

+91 81428 30194

info@twinklechildrenshospital.com

www.twinklechildrenshospital.com

4th Floor; Opposite to Beeranguda Water Tank,
Mumbai Highway, Kakateya Nagar, Ashok Nagar,
Hyderabad, Telangana 502032

CHILD SPECIALIST CARE
Expert care for your child's
healthy tomorrow

COMPASSIONATE & PERSONALIZED CARE
Caring hands, personalized
attention

VACCINATION & WELLNESS
Vaccines today for a
healthier future

SAFE, FRIENDLY & HYGIENIC ENVIRONMENT
Safe environment for
happy healing

GSTIN: 36ABCD1234E125

CONSULTATION RECEIPT

Receipt No: RCP00001

Date: 31 Jul 2026 12:12

Patient: Baby Anika (4M)

Patient No: P00001

Doctor: Dr. A. Kumar

Token: 1 Visit: V00001

PARTICULARS

AMOUNT

Consultation fee — Pediatrics

300.00

RECEIVED ₹300.00

Paid by Cash

Rupees Three Hundred only

Consultation services. Fees once paid are not refundable.

DUPLICATE

1 of 1

Close

Print

Print sends it. **Close** goes back. If no printer is set up the application says so plainly instead of failing, and you can still preview.

Twinkle Children's Hospital

40

Document	Number	Print it from
Tax invoice (medicines)	INV00001	Counter · Reports day book · patient record
Consultation receipt	RCP00001	OPD tile · patient record
Prescription	V00001	Consultation · OPD tile · patient record

Anything can be reprinted at any time. A reprint is stamped **DUPLICATE** so it cannot be mistaken for the original.

14. Worked examples

Real situations, start to finish, with the numbers.

Example 1 — Nine tablets out of a strip of ten

A mother wants only nine Paracetamol. A strip holds ten and costs ₹30.

Step	What you do
1	Pharmacy counter → type <code>parac</code>
2	Click Paracetamol 500mg . It says "590 tablets in stock · ₹3.00 each"
3	How many <code>9</code> , of <code>tablets</code>
4	Add to bill

What you get

- Bill line: `0 × 10 TAB + 9 tablets`, amount **₹27.00**
- Shelf: 590 → 581 tablets
- The opened strip has one tablet left in it, and the next customer gets that first
- The search box empties, ready for the next medicine. The bill keeps the line

Why not ₹270? Because the medicine says ten tablets per pack, so a tablet costs ₹3.00, not ₹30.00. If it charged ₹270, that medicine has **Units in one pack** set to 1 — see task N.

Example 2 — Two whole strips

Same medicine, but the customer wants two full strips.

How many 2 , of strips of 10 → 20 tablets, ₹60.00.

A full strip always costs exactly the MRP printed on it. Two strips is exactly twice that — never ₹59.90 or ₹60.10.

Example 3 — The medicine is found but has no stock

A prescription asks for Cetirizine syrup. The counter shows it, greyed, with "out of stock" — but you can see the bottles on the shelf.

This is the commonest situation in a new system: the delivery arrived and nobody entered it.

Step	What you do
1	Click the medicine anyway. The summary reads "Cetirizine 10mg · out of stock"
2	Click Stock came in — add it
3	Packs on the shelf 5 , MRP per pack 85.00
4	Leave batch and expiry blank if you do not have them
5	Add to shelf

What you get

- Five bottles on the shelf, sellable immediately
- A batch numbered CTR-260727-1432 — allocated, not printed on the pack
- A line in **Reports** → **Stock to reconcile** until the supplier bill arrives

You are back on the same bill. Nothing was lost.

Example 4 — The doctor prescribes something you do not stock

Dolo syrup, which the shop has never carried.

The doctor types the name and **does not** pick anything from the list. The hint reads "Not in our pharmacy — it will be written on the prescription only".

It is printed on the prescription and **never added to your medicine records**. At the counter, **Load prescription** names it: "Not added: Dolo Syrup". Tell the parent to buy it outside.

Example 5 — A course that spans two batches

Twenty tablets. The oldest batch holds seventeen.

You type 20 . The counter splits it and tells you:

Paracetamol: 20 from 2 batches — 15 from PC1234, 5 from PC1180.

Two bill lines, because the batch number of what you actually hand over must be printed. The split falls at **fifteen, not seventeen**, so one whole strip is charged at the printed price and only five are loose — ₹116.65, the same as if one batch had covered it.

On the printed bill both lines sit under one **Paracetamol** heading, so it does not read as two separate items.

Example 6 — Nine tablets, but only six left

You type 9 . Nothing is added, and it says:

Only 6 tablets of Paracetamol 500mg left to sell.

Sell the six, or add stock. It never quietly sells you fewer than you asked for.

Example 7 — A sealed pack that cannot be split

ORS sachets, sold as a box of ten, "Sell loose units" unticked.

You type 7 and it refuses:

ORS Powder is not sold loose — it goes out in whole packs of 10. Enter 10 for 1 pack.

Untick that box on any medicine that must leave the shop whole.

Example 8 — A whole visit

Aarav, four years old, fever.

Step	Screen	What happens
1	OPD → + New visit	Booked, token 4
2	Tile → Fee	₹300 taken, receipt printed
3	Tile → Consult	Fever, viral fever, three medicines prescribed
4	Counter → Load prescription	Paracetamol added; Cetirizine reported <i>no stock</i> ; Dolo reported <i>not added</i>

Step	Screen	What happens
5	Stock came in — add it	Five bottles of Cetirizine on the shelf
6	Load prescription again	Both now on the bill
7	Save & print bill	9 tablets + 1 bottle = ₹112.00

The parent buys the Dolo syrup outside. Everything else is on one bill, with batch numbers and expiry printed on it.

15. What the system will and will not do

Worth reading once. It is the difference between trusting a number and checking it.

Situations it handles for you

Situation	What happens
A child needs 9 tablets from a strip of 10	Priced per tablet, nine come off the shelf, and the bill shows 0 × 10 TAB + 9
A course needs 20 but the oldest batch holds 15	Split across two batches, two lines, both batch numbers on the bill. Nearest expiry goes first
A course needs 20 and there are only 12	It bills 12 and says "Short: Amoxicillin (12 of 20)". It never quietly bills less without telling you
A strip of 15 and a strip of 10 of the same drug	Each batch prices against the pack it actually came in. Old stock is never repriced
A medicine is prescribed that you do not stock	Written on the prescription, named at the counter, never added to your records
A medicine has run out mid-queue	Add it from the counter — see section 10
Stock is expired	Never dispensed. It stays on the shelf listing so you can see it and return it
A sealed pack that cannot be split	Untick Sell loose units and the counter insists on whole packs, telling you the number to type
A Schedule H1 medicine	Cannot be saved without the prescribing doctor. It goes in the register automatically
Two people on one phone number	Both appear when you search. You pick which child is here
Two patients registered one after the other	The screen clears after each save, so the second is a new record. It cannot overwrite the first

Situation	What happens
A batch expiring within a month	Still sold first, and the counter says so before the customer leaves — batch, month and days remaining
The same supplier bill loaded twice	Refused. The invoice number is what stops it
The same batch delivered again	Added to what is there. Never replaced
The count on the shelf is wrong	Correct it with a reason. Every correction is recorded with was, now and why
A printer is not attached	Everything previews on screen first. Nothing is lost by not printing
The power goes off mid-bill	Nothing is half-saved. A bill either exists completely or not at all
Something unexpected fails	It says so in plain words and keeps running. Your saved data is safe

Situations it does not handle — check these yourself

Situation	What you need to know
A customer returns medicine	There are no returns in this version. Do it on paper and correct the stock count with reason <i>Other</i> and a note
Part payment, or paying later	Not supported, on purpose. A bill is paid in full when it is saved
Purchases balanced against sales	Will not tie out while anything sits in Reports → Stock to reconcile . That list is the gap, and it is deliberate
The rate paid on counter-added stock	Defaults to zero, so anything added that way looks like pure margin until you fill the real rate in
No discounts	The counter charges MRP. There is no discount box, by choice
A batch more than a month from expiry	Sold first, correctly, but not flagged. Watch Expiring soon for what to return
Two people billing at once	One PC, one till. This is not built for two counters on the same data
Changing units per pack after stock exists	Offered as a re-count, and you should accept it. Declining leaves the shelf being sold by the pack
A medicine deleted or made inactive with stock on it	It disappears from the counter but the stock is still counted in reports
Anyone can do anything	There are no logins or permissions. Everyone using the PC has the same rights

The one number to sanity-check daily

Reports → **the totals along the top**. The day's takings in the drawer should match pharmacy sales plus fees collected. A bill line can no longer be repriced, so a difference means cash was taken out or a bill was not saved — both worth finding on the day rather than at month end.

16. When something goes wrong

The application does not close on an error

If something unexpected happens it tells you in plain words, writes the details to the log, and **keeps running**. A half-typed bill is not lost. You will see a message like:

Taking the fee could not be completed. The change could not be saved — it may already exist, or a required field is missing. Nothing was changed.

Things it refuses on purpose

It says	Why
"Only 3 × 10 TAB + 4 left of ..."	You cannot sell stock you do not have
"Batch ... expired on ..."	Expired medicine cannot be dispensed
"3 people match that. Select which one"	Siblings share a phone; the wrong child is worse than a second click
"This patient has visits on record"	Deleting them would orphan their bills
"Bill ... was already received on ..."	Importing twice would double your stock
"No MRP" during import	The counter prices from MRP and cannot sell without one
"Batch number is required"	It has to appear on the bill by law

Your data

Everything is in one file: `C:\ProgramData\TwinkleHMS\twinkle.db` . Copy that file and you have copied the clinic.

Backups are automatic — one per day into `C:\ProgramData\TwinkleHMS\backups` , keeping the last 14. That protects against mistakes, **not against the PC dying**. Copy the folder to a pen drive weekly.

The log is at `C:\HMS\Logs` , one file per day, 30 days kept. Settings has an **Open log folder** button and shows the exact path. Send that day's file when reporting a problem.

Changing where things are kept

`appsettings.json` sits next to the application and can be edited in Notepad. Restart the application afterwards.

```
{
  "LogDirectory": "C:\\HMS\\Logs",
  "DatabasePath": null,
  "BackupsToKeep": 14,
  "LogDaysToKeep": 30
}
```

Setting	Notes
LogDirectory	Where daily logs are written
DatabasePath	Full path to the database file. <code>null</code> uses <code>C:\ProgramData\TwinkleHMS\twinkle.db</code>
BackupsToKeep	Daily database backups kept before the oldest is deleted
LogDaysToKeep	Days of logs kept

Use double backslashes in paths, as above.

If the folder cannot be written to — a locked-down PC, or a network drive that is offline — the application does **not** fail. It falls back to `C:\ProgramData\TwinkleHMS\logs` , and the first line of the log says which folder it wanted and which it used. The same applies to a settings file with a typo in it: built-in defaults are used and the log says so.

Not in this version

Sales returns and credit notes · purchase returns · inter-state IGST · e-invoicing · multi-terminal use · the Schedule X narcotic register.

Credit and part payment are not missing — they are deliberately absent. The clinic settles every bill in full at the counter, so there is no outstanding balance anywhere in the system and nothing to reconcile.

The GST arithmetic and the invoice layout are correct for a retail counter, but this is not a certified e-invoicing integration. Have a CA review the GST summary before it feeds a return, and confirm register formats with your local drug inspector.